15.6 Placental Abruption*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "Severe constant pain in my abdomen." **Objective** • Rigid, tender uterus • Dark vaginal bleeding • Pain 9/10	**ACTUAL** Acute pain related to retroplacental hemorrhage as evidenced by constant abdominal pain and a rigid uterus.	• Pain controlled.	• Give analgesia as prescribed. • Prepare for urgent delivery. • Feel uterine tone regularly.	• Met when pain is controlled.
Subjective *No specific complaint* **Objective** • BP 90/60 mmHg, HR 120/min • Hemorrhage may be concealed	**ACTUAL** Inadequate fluid volume related to hemorrhage as evidenced by hypotension and tachycardia.	• Vital signs stable; urine output adequate.	• Insert two large-bore cannulas. • Give fluids and blood as prescribed. • Check clotting for DIC. • Measure urine hourly.	• Achieved when she is stable.
Subjective *No specific complaint* **Objective** • Abnormal fetal heart pattern	**RISK** Risk for impaired maternal-fetal dyad as evidenced by risk factors: placental separation.	• Prompt, safe delivery.	• Keep the fetal heart on continuous monitoring. • Call the obstetric and neonatal teams immediately. • Prepare for urgent cesarean birth.	• Outcome reached when delivery is safe.

* **References for this care plan:** [35], [47], [103]. Full entries are listed in the References section of the CPMN book.